

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO.

Member Information

Member: Odessa J. Pemberton DOB: 1956-04-17 (age 70) Member ID: MBR-30418

Sex: F

Requesting Provider

Practice: Riverbend Cardiology & Imaging

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-10-29

Level of care requested: INPATIENT

Made-up data for a software prototype. Not a real patient.

Cardiac MRI report

RIVERBEND IMAGING CENTER

CARDIAC MRI (CMR) - FUNCTION, PERFUSION-FREE, LGE

Exam date: 09/10/2026 Pt: Pemberton, Odessa J. DOB 04/17/1956

Referring: N. Castellanos, MD Indication: NICM, tissue characterization pre-ICD

Volumes/function (SSFP, short-axis stack):

LVEDV 182 mL (i 101 mL/m²) LVESV 113 mL LVSV 69 mL

LVEF 38%

RVEF 49%

LGE: no myocardial late enhancement.

Other: mild biatrial enlargement, trivial MR.

IMPRESSION:

1. Mildly dilated LV with mildly-moderately reduced systolic function, LVEF 38%.
2. No scar/fibrosis.
3. Compared with TTE 05/18/2026 (LVEF 32%), LV function appears improved, though modalities differ.

Dictated: S. Achebe-Larkin, MD 09/11/2026

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Cardiology consultation

RIVERBEND CARDIOLOGY - CONSULT

Date: 09/02/2026

Odessa Pemberton, 70F, ref'd by PCP for ICD consideration

HPI: NICM dx 10/2025 after presenting w/ DOE. Cath 10/2025 normal coronaries. On GDMT since 11/2025. Now walks 1/2 mile w/ mild dyspnea, limited on stairs. No syncope. Some fatigue.

Data: TTE 05/18/26: LVEF 32%.

A/P: NICM, NYHA II, LVEF 32% after >6 mo GDMT -> meets criteria for primary prevention ICD.

- Order CMR for scar assessment prior to implant.
- SDM visit.
- Refer to Dr. Ibsen (EP) for implant.

N. Castellanos, MD

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Shared decision making note

SHARED DECISION MAKING - IMPLANTABLE DEFIBRILLATOR

Encounter date: 09/04/2026

Patient: Odessa Pemberton

Clinician: Bea Thornquist, NP (Riverbend HF program)

Decision aid: evidence-based ICD decision aid, booklet version, reviewed in full with patient and son.

Topics: benefit for SCD prevention, shock risk, complications, living with device, deactivation, option of waiting for further recovery on meds.

Pt would like to proceed if testing confirms she still qualifies.

B. Thornquist NP

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Echocardiogram report

Riverbend Cardiology - Echo Lab

Date: 05/18/2026

Pemberton, Odessa J

LVEF (biplane): 32%

LVIDd 5.8 cm. Diffuse hypokinesis.

Grade I DD. Mild MR. Normal RV.

Impression: moderately reduced LV function, LVEF 32%. Prior 10/2025: 25%.

Read by: J. Pruett, MD

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Medication list

Pemberton, Odessa J - Medication Profile - printed 09/15/2026

- Entresto 49-51 mg BID Start 11/03/2025
- Metoprolol succinate 100 mg Start 11/03/2025
- Spiroinolactone 25 mg Start 11/03/2025
- Jardiance 10 mg Start 12/2025
- Furosemide 20 mg PRN Start 11/2025
- Alendronate 70 mg weekly Start 2021
- Calcium/vit D OTC

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EP pre-operative note

RIVERBEND CARDIOLOGY - ELECTROPHYSIOLOGY

Pre-operative note 09/15/2026

Patient: PEMBERTON, ODESSA J MBR-30418

Surgeon: Gunnar Ibsen, MD

Procedure: single chamber ICD (CPT 33249), elective inpatient, admit 10/29/2026.

Indication: NICM, LVEF 32% (TTE 5/18/26), NYHA II, GDMT since 11/2025, SDM completed 09/04.

No MI, PCI or CABG history.

Expected LOS: 2 midnights. Factors: osteoporosis with prior vertebral fx and fall risk, chronic anemia (Hgb 10.4) needing post-op Hgb check, lives alone and needs PT eval before d/c.

Contraindications: none - no dementia, no life-limiting illness, no AF, clinically stable.

G. Ibsen MD

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Laboratory results

RIVERBEND LAB SERVICES 09/12/2026
PEMBERTON, ODESSA
Na 139 K 4.5 Cr 0.9 eGFR 66
Hgb 10.4 L MCV 81 L Ferritin 22 L
Plt 251 INR 1.0
NT-proBNP 610 H

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